

For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

FORECOX-TRAC Film Coated Tablet

Rifampicin 150 mg, Isoniazid 75 mg, Pyrazinamide 400 mg & Ethambutol Hydrochloride 275 mg Tablets

Composition:

Each film-coated tablet contains:

Rifampicin BP	150 mg
Isoniazid BP	75 mg
Pyrazinamide BP	400 mg
Ethambutol Hydrochloride BP	275 mg

Description:

Light buff coloured biconvex, capsule shaped film coated tablets with plain surface on both the sides.

Pharmacodynamics Effect

Rifampicin

Rifampicin is a rifamycin antibiotic. Rifampicin exerts, both in vitro and in vivo bactericidal effects on Mycobacterium tuberculosis. It also exhibits variable activity against other atypical species of Mycobacterium.

In vivo rifampicin exerts its bactericidal effect not only on micro-organisms in the extracellular spaces but also on those located intracellularly.

Rifampicin inhibits the DNA-dependent RNA polymerase of sensitive bacterial strains, but without affecting the host enzymatic systems.

Isoniazid

Isoniazid exerts a bactericidal effect mainly on rapidly growing populations of Mycobacterium tuberculosis. Its mechanism of action is probably based chiefly on inhibition of mycolic acid synthesis, mycolic acid being an important constituent of the mycobacterial cell wall.

Pyrazinamide:

The exact mechanism of action is unknown. In vitro and in vivo studies have demonstrated that pyrazinamide is only active at a slightly acidic pH (pH 5.5).

Ethambutol:

The mechanism of action is not fully known. It diffuses into mycobacteria and appears to suppress multiplication by interfering with RNA synthesis. It is effective only against mycobacteria that are actively dividing.

Pharmacokinetics:

Following a single oral dose of four fixed dose combination tablets, each containing Rifampicin 150 mg plus Isoniazid 75 mg plus Pyrazinamide 400 mg plus Ethambutol 275 mg in healthy volunteers the observed pharmacokinetic parameters are as follows:

Rifampicin

Tmax of rifampicin was found to be 1.91 hours, Cmax was 9.83 mcg/mL, AUC_{0-t} was 65.07 mcg·hr/mL, AUC_{0-∞} was 78.17 mcg·hr/mL and T_{1/2} of rifampicin after oral administration of this fixed dose combination was found to be 4.77 hr.

Isoniazid

Tmax of isoniazid was found to be 1.33 hours, Cmax was 5.68 mcg/mL, AUC_{0-t} was 32.25 mcg·hr/mL, AUC_{0-∞} was 33.20 mcg·hr/mL and T_{1/2} of isoniazid after oral administration of this fixed dose combination was found to be 3.32 hr.

Pyrazinamide

Tmax of pyrazinamide was found to be 1.97 hours, Cmax was 31.40 mcg/mL, AUC_{0-t} was 515.43 mcg·hr/mL, AUC_{0-∞} was 541.97 mcg·hr/mL and T_{1/2} of pyrazinamide after oral administration of this fixed dose combination was found to be 10.61 hr.

Ethambutol

Tmax of ethambutol was found to be 3.02 hours, Cmax was 3.26 mcg/mL, AUC_{0-t} was 19.34 mcg·hr/mL, AUC_{0-∞} was 20.20 mcg·hr/mL and T_{1/2} of ethambutol after oral administration of this fixed dose combination was found to be 8.54 hr.

Indications for use:

FORECOX-TRAC is indicated for the initial phase of treating tuberculosis caused by Mycobacterium tuberculosis, according to the guidelines of WHO.

Dosage and administration:

Initial Phase: Daily

Body weight	Dose
30-39 kg	2 tablets
40-54 kg	3 tablets
55-70 kg	4 tablets
> 70 kg	5 tablets

Rifampicin 150 mg/ Isoniazid 75 mg/ Pyrazinamide 400 mg/ Ethambutol Hydrochloride 275 mg Tablets should be taken on an empty stomach (at least one hour before or two hours after a meal). Bioavailability may be reduced if taken with food e.g. to improve gastrointestinal tolerance.

Children: This combination is not prescribed for patients less than 30 kg. Rifampicin 150mg/ Isoniazid 75mg/ Pyrazinamide 400 mg/ Ethambutol Hydrochloride 275 mg Tablets should not be used for intermittent treatment regimens.

Elderly people: No special dosage regimen is necessary, but concurrent hepatic and/or renal insufficiency should be taken into account. Supplementation of pyridoxine (vitamin B₆) may be useful.

Hepatic insufficiency: This combination should be used with caution and under strict medical supervision in impaired liver function. The combination is contraindicated in patients with a history of drug induced hepatitis and in patients with acute liver diseases.

Renal insufficiency: This combination should be used with caution in patients with moderate renal impairment. This combination should be avoided in patients with severe renal impairment.

Contraindications:

- Known or suspected hypersensitivity to rifamycins, isoniazid, pyrazinamide, ethambutol hydrochloride and/or to any of the excipients.
- A history of drug induced hepatitis and acute liver diseases regardless of its origin.
- Porphyria.
- Acute gouty arthritis.
- Severe renal impairment (creatinine clearance < 30 mL/min).
- Concomitant use with voriconazole and protease inhibitors, except ritonavir when given at full dose or 600 mg twice daily.

Warnings and Precautions

Forecox Trac should be withdrawn immediately if severe acute hypersensitivity reactions occur, such as thrombocytopenia, purpura, haemolytic anaemia, dyspnoea and asthma-like attacks, shock or renal failure as these are side effects that rifampicin may provoke in exceptional cases. Patients developing such reactions must never again be treated with rifampicin. Forecox Trac should be withdrawn if other signs of hypersensitivity appear, such as fever or skin reactions. For safety reasons, treatment should not be continued or resumed with rifampicin.

Forecox Trac should be used with care in patients with visual defects. Ocular examinations including acuity, colour discrimination and visual field are

recommended before starting treatment and periodically during treatment, especially if high doses are used. Patients should be questioned at every visit about their vision and advised to discontinue Forecox Trac if a visual disturbance arises pending clinical evaluation.

Forecox Trac is not recommended for children under 8 years of age because of risk of aspiration and because of the ethambutol hydrochloride component. Visual disturbances that may occur as a result of using ethambutol and that require immediate discontinuation of treatment may be difficult to diagnose in young children.

Forecox Trac is not a suitable dosage form for use in the treatment of patients with a body weight of less than 30 kg.

The precautions for the use of Forecox Trac are the same as those that apply for the administration of rifampicin, isoniazid, pyrazinamide and ethambutol as individual medicinal products.

Patients should be advised against interrupting treatment.

Impaired liver function, undernourishment, alcoholism

Rifampicin, isoniazid, pyrazinamide and ethambutol are metabolised in the liver. Elevated transaminase levels, above the upper limit of normal (ULN), commonly occur. Liver dysfunction that may occur in the first few weeks of treatment usually returns to the normal range spontaneously, without interruption of treatment, and usually by the third month of treatment.

With rifampicin, although slight elevations of liver enzymes are common, clinical jaundice or evidence of hepatitis are rare. In patients taking both isoniazid and rifampicin, a cholestatic pattern with elevated alkaline phosphatase suggests that rifampicin is the causative agent, whereas a rise in transaminases may be caused by isoniazid, or rifampicin, or pyrazinamide, or the combination of the three agents.

Patients with impaired liver function should be treated with caution and under strict medical supervision.

In these patients, careful monitoring of liver function, especially serum glutamic pyruvic transaminase (SGPT/ALAT) and serum glutamic oxaloacetic transaminase (SGOT/ASAT) should be carried out prior to therapy and repeated weekly or fortnightly during therapy. If signs of hepatocellular damage occur, Forecox Trac should be withdrawn.

A moderate rise in bilirubin and/or transaminase levels is not in itself an indication for interrupting treatment; rather, the decision should be made after repeating these liver function tests, noting trends in the levels, and considering them in conjunction with the patient's clinical condition.

Interrupting isoniazid treatment is recommended when there is clinical jaundice or transaminases exceeding 3 times the ULN.

Withdrawing rifampicin, pyrazinamide and ethambutol is recommended if liver function does not return to normal or transaminases exceed 5 times the ULN. Use of isoniazid should be carefully monitored in patients with chronic liver disease. Severe and sometimes fatal hepatitis caused by isoniazid may occur and may develop even after many months of treatment. Hepatotoxicity associated with isoniazid therapy (thought to be caused by the metabolite diacetylhydrazine) is rare in patients up to 20 years of age, but more common with increasing age and affecting up to 3% of patients aged over 50 years. The incidence of severe hepatotoxicity can be minimised by careful monitoring of liver function. Patients should be monitored with regard to the appearance of prodromal symptoms of hepatitis, such as fatigue, weakness, malaise, anorexia, nausea or vomiting. If these symptoms appear or if signs of hepatic damage are detected, treatment should be discontinued promptly. Continued use of Forecox Trac in these patients may cause a more severe form of liver damage.

In patients with chronic liver disease, as well as in chronic alcoholics and undernourished patients, the therapeutic benefits of treatment with Forecox Trac must be weighed against the possible risks. If anti-tuberculous treatment is considered necessary, the dosage of rifampicin, isoniazid, pyrazinamide and ethambutol may need to be modified and Forecox Trac should not be used in such

patients because it is only possible to adjust the dosage by administering rifampicin, isoniazid, pyrazinamide and ethambutol separately.

For undernourished or elderly patients supplementation of pyridoxine (vitamin B₆) may be useful, because isoniazid in high doses can lead to pyridoxine (vitamin B₆) deficiency.

Impaired renal function

In severe renal insufficiency, the elimination of isoniazid, pyrazinamide and ethambutol can be delayed leading to a higher systemic exposure, which can result in an increase in adverse events. The combination should be used with caution in patients with moderate renal impairment (creatinine clearance 30-60 mL/min).

Gout

Pyrazinamide and ethambutol should be used with caution in patients with a history of gout. Regular monitoring of serum uric acid should be undertaken. Forecox Trac treatment should be stopped in gouty arthritis.

Haematology

Full blood count should be monitored during prolonged treatment and in patients with hepatic disorders. Rifampicin should be withdrawn permanently if thrombocytopenia or purpura occur. The possibility of pyrazinamide having an undesirable effect on blood clotting time or vascular integrity should be borne in mind in patients with haemoptysis.

Diabetes mellitus

Increased difficulty has been reported in controlling diabetes mellitus when such patients are given isoniazid.

Epilepsy

Patients suffering from convulsive disorders must be kept under special observation during treatment with Forecox Trac because of the neurotoxic effects of isoniazid and ethambutol hydrochloride.

Neuropathy

Caution should be exercised in subjects with peripheral or optic neuritis. Regular neurological examination is necessary with special care in patients with a history of alcohol abuse. Use of pyridoxine (vitamin B₆) may prevent or diminish neuropathy due to isoniazid treatment especially in elderly and in malnourished patients. Pyridoxine should be given in line with official guidelines.

Contraception

Additional non-hormonal means of contraception must be employed to prevent the possibility of pregnancy during treatment with rifampicin.

Alcohol

Patients should abstain from alcohol while receiving treatment with Forecox Trac.

Laboratory tests

Full blood counts, liver function tests (SGPT/ALAT, SGOT/ASAT), renal function tests and monitoring serum uric acid should be performed before treatment and at regular intervals during treatment. Ocular examination is recommended during treatment with ethambutol hydrochloride.

Forecox Trac has minor to moderate influence on the ability to drive and use machines.

Undesirable effects of ethambutol, such as confusion, disorientation, hallucinations, dizziness, malaise and visual disturbances (blurred vision, red-green colour blindness, loss of vision) may impair the patient's ability to drive or operate machinery.

Carcinogenicity, Mutagenicity & Impairment to Fertility

Rifampicin

After oral administration of 100 mg/kg bodyweight rifampicin for 6 months in rats no toxic effects were observed. After chronic administration of 200 mg/kg swelling and hydropic degeneration of the liver were observed.

In monkeys, vomiting, anorexia and weight loss were observed at chronic doses of 105 mg/kg daily.

Because only limited evidence is available on the carcinogenicity of rifampicin in mice and in the absence of epidemiological studies, the carcinogenicity potential of rifampicin in humans cannot be evaluated.

The available studies on mutagenicity indicate an absence of a mutagenic effect.

Rifampicin concentrations in cord blood reach 12–33% of maternal blood concentrations. Teratogenic effects were noted in rodents treated with high doses. Rifampicin 100 to 150 mg/kg daily in rodents have been reported to cause cleft palate and spina bifida.

In rats neither fertility nor perinatal and postnatal development was impaired. Malformation and death in infants born to mothers exposed to rifampicin were reported at the

same frequency as in the general population.

Isoniazid, ethambutol, pyrazinamide

Conventional studies of safety pharmacology, repeated dose toxicity, genotoxicity, carcinogenic potential, and toxicity to reproduction reveal no special hazard for humans at recommended doses.

Interactions with other Medicines

Combination contra-indicated

Use of the following medicinal products concomitantly with Forecox Trac is contra-indicated: voriconazole and proteaseinhibitors, except ritonavir when given at full dose or 600 mg twice daily.

Oral typhoid vaccine might be inactivated by concomitant antibiotic administration.

Combination not advised

Antacids reduce the bioavailability of rifampicin, isoniazid and ethambutol. To avoid this interaction, Forecox Trac should be taken at least 1 hour before antacids.

Use of the following medicinal products concomitantly with Forecox Trac is not recommended: nevirapine, simvastatin, oral contraceptives and ritonavir (when given in low doses as a booster a marked reduction of plasma concentration might occur).

Food with a high content of tyramine or histamine should be avoided. Isoniazid may inhibit monoamine oxidase and diamine oxidase. Intake of food containing tyramine (e.g. cheese, red wine) or histamine (e.g. tuna fish) may lead to headache, palpitations, flushing etc.

Rifampicin can delay the biliary excretion of contrast media during gallbladder radiographic examination.

Microbiological methods used to determine folic acid and cyanocobalamin (vitamin B12) plasma concentrations can not be used during rifampicin treatment as rifampicin is in competition with bilirubin and BSP. To avoid false positive reactions, BSP test should be carried out the morning before rifampicin administration.

Some medicinal products are affected in the opposite direction by rifampicin and isoniazid, e.g. phenytoin, warfarin and theophylline. The net effect cannot be predicted and may change over time.

Corticosteroids can reduce the plasma levels of isoniazid, by increasing its metabolic and/or renal clearance.

Combinations which need precautions of use

Use of the following medicinal products concomitantly with Forecox Trac requires a precaution for use by monitoring specific parameters or through a clinical surveillance: calcium antagonists, class Ia antiarrhythmics (quinidine, disopyramide), oral anticoagulants, azole antifungals (except voriconazole), buspirone, carvedilol (because of its use in cardiac insufficiency and its low therapeutic margin in this indication), immunosuppressive agents (like ciclosporine, tacrolimus, sirolimus), clozapine, corticosteroids, gestrinone, estrogens and progestagens given as hormonal replacement therapy, haloperidol, thyroid hormones, methadone, morphine, efavirenz, propafenone, terbinafine, tiagabine, zidovudine, zolpidem, zaleplon, carbamazepine, phenytoine, theophylline, benzodiazepines, digitalis, dapson, atovaquone, repaglinide or oral antidiabetics of sulfonylurea type, beta-receptor antagonists (if hepatically metabolised such as metoprolol, propranolol), chloramphenicol, clarithromycin, telithromycin, tricyclic antidepressants, p-aminosalicylic acid, cimetidine, mexiletine, nevirapine, fluvastatin, etoricoxib, rofecoxib, imidapril, tropisetron, linezolid.

Rifampicin is the most potent inducer of the cytochrome P450 system (CYP450), notably of the two subfamilies CYP3A and CYP2C, which represent more than 80% of the isoenzymes of CYP450. Thus rifampicin may increase the metabolism of numerous concomitantly administered medicinal products which are metabolised, partially or totally, by these two subfamilies of CYP450. Moreover, rifampicin also induces UDP-glucuronyltransferase, another enzyme involved in the metabolism of several medicinal products. This can result in subtherapeutic plasma levels of the simultaneous administered medicinal products, with a decreased or even a loss of effect. Isoniazid inhibits the metabolism of some medicinal products leading to increased plasma concentrations. Medicinal products that are eliminated by metabolism should only be used concomitantly with Forecox Trac if the plasma concentrations or clinical response/undesirable effects can be monitored and the dose can be adequately adjusted. Monitoring should be performed regularly during Forecox Trac therapy and for 2-3 weeks after discontinuation of the therapy.

Use of the following medicinal products concomitantly with Forecox Trac requires a precaution for use by monitoring specific parameters or through a clinical surveillance: halogenated volatile anaesthetics, glucocorticoids, ketoconazole, phenytoin, pyrazinamide, stavudine, carbamazepine, benzodiazepines, ethosuximide, theophylline, probenecid, sulfapyrazon.

Use of the following medicinal products concomitantly with Forecox Trac requires a precaution for use by monitoring specific parameters or through a clinical surveillance: halogenated volatile anaesthetics, glucocorticoids, ketoconazole, phenytoin, pyrazinamide, stavudine, carbamazepine, benzodiazepines, ethosuximide, theophylline, probenecid, sulfapyrazon.

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Combinations to consider

Rifampicin may reduce the effectiveness of oral contraceptives and patients treated with Forecox Trac should use a non-hormonal method of contraception.

The enzyme inducing effects of rifampicin reach a peak within 10 days and gradually decrease over a period of 2 or more weeks after discontinuation of rifampicin treatment, factors that must be taken into account if the dose of other medicinal products is increased during treatment with Forecox Trac.

Overdosage:

Rifampicin

Symptoms: Gastrointestinal complaints, vomiting, sweating, dyspnoea, seizures, renal failure, liver involvement, impaired consciousness, generalised pruritus. Reddish-orange discoloration of skin and urine, facial oedema. Possibly pulmonary oedema.

Treatment: If authorised, evacuation of the stomach, repeated doses of charcoal. Symptomatic treatment. Dialysis may be required in the event of renal failure.

Isoniazid

Symptoms: Typical symptoms are seizures and metabolic acidosis, ketonuria, hyperglycaemia. In addition, periorbital myoclonus, dizziness, tinnitus, tremor, hyperreflexia, paresthesias, hallucinations, impaired consciousness. Respiratory depression, apnoea. Tachycardia, arrhythmias, hypotension. Nausea, vomiting. Fever, rhabdomyolysis, DIC, hyperglycaemia, hyperkalaemia. Liver involvement.

Doses of isoniazid exceeding 10 mg/kg may adversely affect the nervous system, e.g. in the form of peripheral neuropathy, and thus impair the patient's ability to drive or operate machinery.

Treatment: If authorised, evacuation of the stomach (provided the patient is not experiencing seizures), charcoal. Blood samples must be collected for immediate determination of blood gases, electrolytes, BUN, glucose etc. In the event of seizures and metabolic acidosis, pyridoxine is given at 1 g per g isoniazid. In the event of seizures and unknown dose, 5 g pyridoxine is given iv. In the absence of seizures, 2-3 g pyridoxine is given prophylactically intravenously. Pyridoxine should be diluted to reduce vascular irritation and is administered for 30 minutes via infusion pump or syringe pump. The dose is repeated if necessary. Diazepam potentiates the effect of pyridoxine. A high dose of diazepam can also be tried to combat seizures if pyridoxine is unavailable. In severe cases, respiratory therapy. Correction of metabolic acidosis and electrolyte disturbances. Ensure good diuresis. Haemodialysis or haemoperfusion in the event of extremely severe intoxication. Symptomatic treatment.

Pyrazinamide

Abnormal liver function tests, hyperuricaemia.

Ethambutol

Loss of appetite, gastro-intestinal disturbances, fever, headache, dizziness, confusion, hallucinations.

Side Effects:

The most important adverse effects of rifampicin are hepatotoxicity, particularly cholestatic reactions, and skin reactions. Rifampicin may cause subclinical, unconjugated hyperbilirubinaemia or jaundice without hepatocellular damage, but occasionally causes hepatocellular injury. It can also potentiate the hepatotoxicity of the other anti-tuberculosis medicines.

The most important adverse effects of isoniazid are peripheral and central neurotoxic

effects, and hepatotoxicity. Severe and sometimes fatal hepatitis associated with isoniazid therapy has been reported. The majority of cases have occurred within the first three months of therapy, but hepatotoxicity may also develop after a longer treatment.

The most important adverse effect of pyrazinamide is liver damage, ranging from asymptomatic increases of serum transaminases to symptomatic liver dysfunction, and in rare cases also fatal liver failure.

The most important adverse effect of ethambutol is retrobulbar neuritis with reduced visual acuity. The frequency depends on the dose and duration of therapy. It has been reported in up to 3% of patients receiving ethambutol 20 mg/kg daily. Typical initial signs include impairment of colour vision (red-green blindness) and constriction of visual field (central or peripheral scotoma). These changes are often reversible on discontinuing therapy. To avoid irreversible optic atrophy, visual acuity should be monitored regularly and ethambutol must be discontinued immediately if visual disturbances occur.

Frequencies are defined as very common (≥ 1 in 10), common (1 in 100 to 1 in 10), uncommon (1 in 1000 to 1 in 100), rare (1 in 10 000 to 1 in 1000), very rare (≤ 1 in 10 000), and 'not known' (frequency cannot be estimated from the available data).

Nervous system disorders:

Very common: peripheral neuropathy, usually preceded by paraesthesia of the feet and hands. The frequency depends on the dose and on predisposing conditions such as malnutrition, alcoholism or diabetes. It has been reported in 3.5 to 17% of patients treated with isoniazid. Concomitant pyridoxine administration largely reduces this risk.

Uncommon: headache, lethargy, ataxia, difficulty concentrating, dizziness, seizures, toxic encephalopathy.

Not known: tremor, vertigo, hyperreflexia, insomnia.

Psychiatric disorders:

Uncommon: memory impairment, toxic psychosis.

Rare: hyperactivity, euphoria.

Not known: confusion, disorientation, hallucination.

Gastrointestinal disorders:

Common: diarrhoea, abdominal pain, nausea, anorexia, vomiting.

Rare: erosive gastritis, pseudomembranous colitis.

Very Rare: pancreatitis.

Not Known: metallic taste, dry mouth, flatulence, constipation.

Hepatobiliary disorders:

Very common: transient elevation of serum transaminases.

Uncommon: elevated serum bilirubin and alkaline phosphatases, hepatitis.

Renal and urinary disorders:

Rare: acute renal failure, interstitial nephritis.

Not Known: urinary retention.

Metabolic and nutritional disorders:

Very common: hyperuricaemia, especially in patients with gout.

Very Rare: aggravated porphyria.

Not Known: hyperglycaemia, metabolic acidosis, pellagra.

General disorders:

Very common: flushing.

Common: reddish discoloration of body fluids and secretions, such as urine, sputum, tears, saliva and sweat.

Not Known: allergic reactions with skin manifestations, pruritus, fever, leucopenia, anaphylaxis, allergic pneumonitis, neutropenia, eosinophilia, vasculitis, lymphadenopathy, rheumatic syndrome, lupus-like syndrome, hypotension, shock.

Blood and lymphatic system disorders:

Not Known: anaemia (haemolytic, sideroblastic, or aplastic), thrombocytopenia, leucopenia, neutropenia with eosinophilia, agranulocytosis, blood clotting affected.

Respiratory, thoracic and mediastinal disorders:

Not Known: pneumonitis, dyspnoea.

Musculoskeletal disorders:

Very common: arthralgia.

Not Known: gout.

Skin and subcutaneous tissue disorders:

Common: erythema, exanthema, pruritus with or without rash, urticaria.

Rare: photosensitivity, exfoliative dermatitis, pemphigoid reactions, purpura.

Not Known: Lyell's Syndrome, Stephens-Johnson Syndrome.

Eye disorders:

Common: ocular redness, permanent discoloration of soft contact lenses, visual disturbances due to optic neuritis (retrobulbar neuritis).

Rare: exudative conjunctivitis.

Reproductive system and breast disorders:

Common: disturbances of the menstrual cycle.

Rare: gynaecomastia.

Effects on the ability to drive or operate machinery

Forecox Trac has minor to moderate influence on the ability to drive and use machines.

Undesirable effects of ethambutol, such as confusion, disorientation, hallucinations, dizziness, malaise and visual disturbances (blurred vision, red-green colour blindness, loss of vision) may impair the patient's ability to drive or operate machinery.

Use in Pregnancy and Lactation:

Pregnancy: No adverse effects of isoniazid, ethambutol or pyrazinamide on the fetus have been reported. Use of rifampicin in the third trimester has been associated with postnatal haemorrhages in the mother and infant. The combination should be used in pregnancy only if the benefits are considered to outweigh the risks. If the combination is used in the last weeks of pregnancy, the mother and neonate should receive vitamin K.

Lactating Mothers: Rifampicin, isoniazid, pyrazinamide and ethambutol pass into the breast milk. However, concentrations in breast milk are so low that breast-feeding cannot be relied upon for adequate tuberculosis prophylaxis or therapy for nursing infants. No adverse effects in the baby have been reported.

Storage: Store below 30°C, protected from direct sunlight.

Presentation:

1. 24 X 28's Tablets

2. 10 x 6's Tablets

Blister pack of 60 and 672 tablets which is made up of Amber color PVC/PE/PVDC film and printed Aluminium Foil was later packed in carton together with package insert.

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18/03/2022

MACLEODS



Manufactured in India by:

MACLEODS PHARMACEUTICALS LTD.

Plot No. 25-27, Survey No. 366,

Premier Industrial Estate, Kachigam,

Daman - 396 210 (U.T.)

Off.: Allianta Arcade, 3rd Floor, Marol Church Road,

Near Leela Hotel, Andheri (E), Mumbai - 400 059.

Product Registration Holder:

ZULAT PHARMACY SDN. BHD.

No 23 & 23A, Jalan Bandar 3,

Taman Melawati, 53100,

Kuala Lumpur, Malaysia